

Lakeview Medical Center - Discharge Packet

Patient: Ana Rivera | MRN: LMC-482913 | DOB: 1979-04-18 | Packet finalized: 2026-08-14 14:40

Discharge summary

Admission date: 2026-08-10. Discharge date: 2026-08-14. Primary diagnosis: community-acquired pneumonia with acute kidney injury, improving. Secondary diagnoses: type 2 diabetes, hypertension, and medication-associated hyperkalemia. Allergy: penicillin - rash. Pending test: blood culture final read expected 2026-08-16.

Condition at discharge: stable on room air. Home health nurse visit is authorized for 2026-08-15. Medication reconciliation on page 3 controls over the ED medication list and any draft discharge instruction footer. Renal dosing should be reviewed again after the outpatient BMP.

Source-state notice: FINAL medication reconciliation signed 14:40 controls. ED medication list and draft footer are historical. Do not restart lisinopril until follow-up BMP is reviewed.

Follow-up need	Due	Owner	Status	Instruction
BMP lab draw	2026-08-17 08:00	Home health	Scheduled	check K and creatinine
Primary care	2026-08-19 10:30	Dr. Shah	Scheduled	review antibiotics
Cardiology	2026-08-25 09:00	Dr. Kim	Scheduled	11 days after discharge
Blood culture final	2026-08-16	LMC lab	Pending	call if positive

Footer draft 08:15 said cardiology in 2 weeks; scheduled referral sheet controls with 2026-08-25.

Lab Trends - CBC / CMP / Coagulation

Flags and units are part of the record. Hemolyzed/canceled specimens are excluded from trend interpretation.

Analyte	Ref range	Unit	08-10 06:20	08-12 05:50	08-14 06:05
WBC	4.0-11.0	K/uL	14.2 H	9.8	7.6
Hemoglobin	12.0-16.0	g/dL	10.8 L	10.1 L	10.4 L
Platelets	150-400	K/uL	412 H	390	366
Creatinine	0.6-1.2	mg/dL	1.9 H	1.5 H	1.2
Potassium	3.5-5.1	mmol/L	5.8 H	hemolyzed	4.7
Glucose	70-110	mg/dL	188 H	142 H	126 H
INR	0.9-1.1	ratio	1.0	canceled	1.1

Footnotes: 08-12 potassium is hemolyzed and excluded.
08-12 INR was canceled and should not be carried forward.
Hemoglobin remains low at discharge.

Creatinine sparkline



Final Medication Reconciliation

Signed by pharmacist M. Patel at 2026-08-14 14:40. This page controls over ED medication list.

Medication	Home dose	Discharge dose	Route	Frequency	Action	Note
Metformin	500 mg	500 mg	PO	twice daily	continue	take with meals
Lisinopril	20 mg	HOLD	PO	do not restart	stop	restart only after BMP
Azithromycin	-	250 mg	PO	daily x3 days	new	finish course
Insulin glargine	18 units	14 units	SQ	nightly	change	dose reduced
Ibuprofen	400 mg	STOP	PO	none	stop	avoid with kidney injury

- ☒ Continue: Metformin
- ☒ Change: Insulin glargine
- ☒ Stop: Lisinopril
- ☒ New: Azithromycin
- ☐ Restart lisinopril today

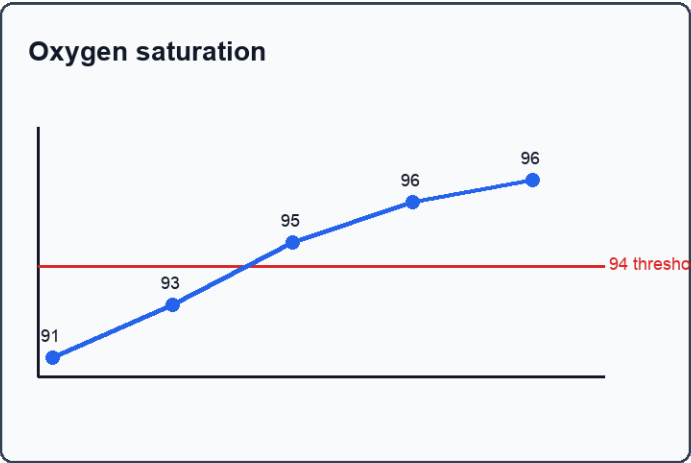
20 mg daily

Pharmacist note: ~~ED list showed~~ lisinopril 20 mg daily and ibuprofen PRN, but both are inactive at discharge. Do not copy ED list into final medication section.

Nursing MAR and Vitals Timeline

Grid states: A=administered, H=held, R=refused, M=missed. Held doses are not administered.

Medication	06:00	12:00	18:00	22:00	Comment
Azithromycin		A			given after lunch
Insulin lispro	A	H	A		12:00 held: glucose 88
Acetaminophen		R		A	refused noon dose
Lisinopril	H	H	H	H	held all day
Metformin	A		A		continued



Blood pressure

Time	BP	Pulse
06:00	132/84	88
12:00	118/72	82
18:00	126/78	80
22:00	124/76	78

Referral Sheet and Draft Footer

Referral sheet signed 2026-08-14 14:46. These scheduled dates control over draft footer text.

Appointment	Date/time	Location	Status	Instruction
Home health nurse	2026-08-15 09:00	home	authorized	med check and vitals
BMP lab draw	2026-08-17 08:00	LMC Lab	scheduled	bring lab slip
Primary care	2026-08-19 10:30	Clinic A	scheduled	review antibiotics
Cardiology	2026-08-25 09:00	Heart Center	scheduled	11 days after discharge

Authorization stamp: home health visit approved once for 2026-08-15. Additional visits require PCP order.

Draft footer from 08:15: cardiology in 2 weeks; resume home blood pressure pill. Superseded by final medication reconciliation and signed referral schedule.